HOSPITAL DISCHARGE SUMMARY

Synthetic case: DEMO-CARD-6024 | Admission: 2026-05-02 | Discharge: 2026-05-07
Facility: Fictional Lakeside General | Service: Cardiology

PRESENTING CONCERN

Progressive exertional breathlessness and ankle swelling over approximately 10 days.
The synthetic history notes a 2.1 kg weight increase compared with a home log entry from two weeks earlier. No chest pain was documented at triage.

DISCHARGE DIAGNOSES

1. Acute decompensated heart failure with reduced ejection fraction, documented in the discharge diagnosis list; echocardiogram during admission reported LVEF 35%.
2. Hypertension, history documented; inpatient readings improved before discharge.
3. Chronic kidney disease, stage 3a per prior chart history; current creatinine is listed below.

RELEVANT HISTORY AND ALLERGIES

Prior myocardial infarction is listed in a 2022 outside note; no event during this admission.
Allergy list: lisinopril - persistent dry cough (reaction recorded as intolerance in source).
No contrast allergy is recorded in the provided record.

HOSPITAL COURSE

The team documented volume overload on presentation. Intravenous diuretic treatment was given. Breathing and peripheral edema improved. No mechanical ventilation was required. Telemetry showed no sustained ventricular arrhythmia in the documented period. Discharge weight: 74.8 kg.

HOSPITAL DISCHARGE SUMMARY - CONTINUED

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SELECTED DISCHARGE LABORATORY RESULTS

Sodium 136 mmol/L (135-145); potassium 4.4 mmol/L (3.5-5.1); creatinine 1.42 mg/dL (0.60-1.10), reported high; estimated GFR 52 mL/min/1.73m2 (>=60), reported low.
The summary reproduces the stated laboratory ranges and flags; it does not independently interpret renal function or medication suitability.

MEDICATION RECONCILIATION - TAKE AT DISCHARGE

Furosemide 40 mg by mouth each morning. The note says this replaces the prior 20 mg daily dose.
Carvedilol 6.25 mg by mouth twice daily with food.
Empagliflozin 10 mg by mouth once daily; newly listed at discharge.
Aspirin 81 mg by mouth once daily; continued from home list.
Potassium supplement: held at discharge; the record says to review at follow-up.
Lisinopril: remains discontinued because the documented cough reaction; no restart instruction.

FOLLOW-UP

Cardiology appointment requested within 7 days; appointment date is not present in this document.
Basic metabolic panel recommended in 3-5 days. Daily weights and written return precautions were discussed per note. These actions are documented instructions; completion is not confirmed.

END OF REPORT - Synthetic educational test fixture; not a real patient record.